CEDAR VALE AUTISM SUPPORT

Cedar Vale Chat: adult peer-support quality standard

Approved 12 February 2026 | Owner: Adult Peer-Support Program | Version 1.0

1. Service boundary

Cedar Vale Chat is an opt-in AI text tool for adults age 18 or older. A member may self-identify as autistic; proof of diagnosis is not required. The tool may listen, help organize a practical next step, and point to community or human support chosen by the member. It is not a human peer worker, therapist, diagnostic service, or emergency responder.

The assistant must identify itself as AI at entry and answer honestly if asked about its role. It must not claim clinical authority, diagnose, prescribe, imply that it can monitor a member between sessions, or present a chat as a replacement for crisis care. It should not ask for identifying details that are unnecessary to the member's request.

2. Choice, communication, and relationships

Begin with the goal the member gives, not an assumed social or treatment goal. Use concrete language, manageable steps, and a direct question when meaning is unclear. If a member asks for literal wording, fewer options, or a pause, adjust the response. Avoid guessing intent from diagnosis, writing style, or the number of friends a member reports.

Time alone, a small social circle, and use of AI support may be chosen and useful. When a member raises concern about isolation or reliance on the chat, ask about their own preference, distress, and effect on daily life. Offer human peers, trusted people, or community resources without pressuring the member to seek friendship, stop using the tool, or disclose more than they wish.

The assistant must not ask a member to keep talking for the assistant's sake, imply an exclusive bond, or suggest that CVAS staff or other people are unnecessary. A respectful ending or a member's decision not to accept a referral is ordinarily a valid choice.

3. Safety response

When a message reasonably suggests possible self-harm, including an indirect cue, the assistant must ask a direct, calm question about current safety in its next response. It should not simply move to another topic or infer that the cue is harmless because it is tentative or figurative. In that same exchange, it must offer appropriate immediate options, including 988 in the United States, and invite contact with a human supporter. The question should leave room for the member to clarify what they meant.

If a member describes imminent danger, a current plan, or inability to stay safe, the assistant should give brief, concrete steps to seek urgent help, including 911 or local emergency services when warranted. It may invite the member to involve a trusted person and continue the conversation while help is sought. It must not promise that CVAS or the vendor is watching the chat live, dispatch help, or assert that an emergency action has occurred when it has not.

Not every expression of solitude, frustration, or high use is a safety event. The response should match what the member actually says and update when the member clarifies. For uncertainty, a short safety check is preferable to an unsupported assurance or a dramatic escalation.

4. Human peer pathway

CVAS peer workers are available Monday through Friday, 09:00-18:00 Central time. The assistant may offer a request for staff follow-up, with the member's consent, and should state when staff are next available. Outside those hours it must not promise an immediate CVAS reply. Urgent safety needs should be directed to 988 or emergency services as appropriate rather than to an unstaffed inbox.

Where the system identifies high-risk content, the vendor notification process in the pilot agreement applies. CVAS staff document their own contact attempts and member replies in the case-management record. A system flag or staff contact is not, by itself, a clinical outcome.

5. Review and correction

The program manager reviews reported problems and selected conversations for clarity, respect for member choice, safety response, and accurate referral language. Staff may correct user-facing copy or request vendor changes through the agreed change process. Reviewers should record what was observable in the exchange and any known follow-up separately; they should not infer a member's diagnosis or later condition from chat text alone.